

PATIENT IDENTIFICATION

Name: Priya Singh

Age: 42

Gender: Female

Date of Birth: 1984-09-25

Patient ID: PT-55290

CHIEF COMPLAINT

Patient reports a severe, productive cough, mild fever, and shortness of breath that has worsened over the past 4 days.

MEDICAL HISTORY

- Mild intermittent asthma (Diagnosed in childhood)
- Seasonal allergic rhinitis
- Appendectomy (2010)

ALLERGIES

- Penicillin (Causes hives and facial swelling)
- Peanuts (Mild gastrointestinal distress)

CURRENT MEDICATIONS

- Albuterol sulfate HFA inhaler (2 puffs every 4-6 hours as needed)
- Loratadine 10 mg once daily (during allergy season)

VITALS AT TRIAGE

- Blood Pressure: 118/76 mmHg
- Heart Rate: 92 bpm
- Temperature: 100.5 F (Low-grade fever)
- Respiratory Rate: 22 breaths/min
- Oxygen Saturation (SpO2): 94% on room air
- Weight: 62 kg

- Height: 1.63 meters

LABORATORY & IMAGING RESULTS (Date: 2026-05-03)

- Chest X-Ray: No focal consolidation, pneumothorax, or pleural effusion. Lungs are clear, ruling out pneumonia.
- Complete Blood Count (CBC): White Blood Cell (WBC) count slightly elevated at 11,500/mcL, suggesting a mild infection.

PHYSICIAN NOTES & PLAN

Patient is experiencing an acute exacerbation of asthma likely triggered by a viral upper respiratory infection. Lungs exhibit widespread wheezing upon auscultation, but X-ray confirms no pneumonia.

1. Administered one nebulized Albuterol treatment in the clinic; SpO2 improved to 98%.
2. Prescribed a 5-day course of oral Prednisone 40 mg daily to reduce airway inflammation.
3. Advised to increase the use of the Albuterol rescue inhaler to every 4 hours for the next 48 hours.
4. Instructed to go to the emergency room immediately if shortness of breath worsens or if lips turn bluish.